5.4 Increased Intracranial Pressure*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • Headache getting worse **Objective** • GCS falling • Projectile vomiting • BP 180/70 mmHg, HR 52/min (widening pulse pressure)	**ACTUAL** Acute confusion related to raised intracranial pressure as evidenced by declining GCS and decreasing responsiveness.	• ICP and cerebral perfusion pressure within the prescribed limits.	• Report any sign of rising ICP at once. • Keep the head of the bed at 30 degrees, head in midline. • Avoid what raises ICP: long suction passes, coughing, straining. • Give osmotic therapy as prescribed. • Keep temperature and glucose normal.	• Outcome reached while neurological status is stable.
Subjective *No specific complaint* **Objective** • Irregular breathing • SpO_2 90%	**ACTUAL** Ineffective breathing pattern related to pressure on the respiratory center as evidenced by irregular respirations and low SpO_2.	• SpO_2 94% or more with a regular breathing pattern.	• Keep the airway open and give oxygen as prescribed. • Keep RR, pattern and SpO_2 under continuous watch. • Prevent hypercapnia, as it raises ICP. • Prepare for intubation if ordered.	• Considered met at target SpO_2.
Subjective *No specific complaint* **Objective** • Reduced consciousness • Vomiting	**RISK** Risk for aspiration as evidenced by risk factors: decreased consciousness and vomiting.	• Airway clear, no aspiration.	• Nurse on the side unless contraindicated. • Keep suction at the bedside. • Nil by mouth until swallowing has been assessed.	• Goal reached if aspiration does not occur.

*References for this care plan: [13], [35], [37], [124], [128]. Full entries are listed in the References section of the CPMN book.